

MILPERSMAN 1300-1400

LIMITED DUTY AND PHYSICAL EVALUATION BOARD

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References	(a) DoD Instruction 1332.45 of 30 July 2018 (b) OPNAVINST 1300.20A (c) DoD Instruction 1332.18 of 10 November 2022 (d) 10 U.S.C. (e) SECNAV M-1850.1, Department of the Navy Disability Evaluation System Manual, September 2019 (f) OPNAVINST 1300.21 (g) NAVMED P-117, Manual of the Medical Department (MANMED) (h) Manual for Courts-Martial, January 2024 (i) OPNAVINST 1220.1F
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1. **Purpose.** This article provides the procedures for the assignment, accountability, follow-up care, and disposition of active duty and Training and Administration of the Reserves officer and enlisted personnel to or from a limited duty (LIMDU) status or physical evaluation board (PEB) for medical reasons.

2. **Definitions**

a. **NAVMED 6100/5 Abbreviated Medical Evaluation Board Report (AMEBR).** A detailed summary of a Service member's medical condition(s) dictated by the attending physician. An AMEBR is required to be submitted electronically to the Service member and Marine Readiness Tracker (SMART) and is used to recommend a member being placed on or extended on a period of LIMDU. An AMEBR is also used to place a Service member on LIMDU for referral to the Disability Evaluation System (DES).

b. **NAVMED 6100/6 Return of a Patient to Medically Unrestricted Duty from Limited Duty.** A form used to return a Service member to medically unrestricted duty after a period of

LIMDU and to identify any duty or assignment limitations. [NAVMED 6100/5](#) and [NAVMED 6100/6](#) are completed electronically in the appropriate LIMDU tracking system (e.g, SMART).

NOTE: An AMEBR is not to be confused with a special function Electronic Medical Board Report (Emebr) which is a form submitted in SMART specifically to refer a Service member to the DES.

c. **Deployability Category (DCAT) Code.** DCAT identifies a Service member's deployability status. DCAT delineates the deployability category assignment of varying conditions (e.g., medical, legal, humanitarian assignment, pregnancy.) into categories reportable to leadership and visible to personnel with an official need to know (Navy Personnel Command (NAVPERSCOM) detailers and other staff, etc.).

d. **Deployability Coordinator.** Report and track medically and administratively restricted Service members within their area of responsibility (AOR).

(1) **Command Deployability Coordinator (CDC).** Per references (a) and (b), every command must appoint, in writing, a single point of contact to act as the CDC. Commands with 50 or more personnel in a medically restricted status will appoint a CDC as a primary duty.

(2) **Medical Deployability Coordinator (MDC).** Per references (a) and (b), every Navy Medicine Readiness and Training Command (NMRTC) will appoint an MDC to liaise with CDCs and assist with appointments and medical concerns affecting personnel in a medically restricted status.

(3) **Regional Deployability Coordinator (RDC).** Every type commander (TYCOM) will appoint an RDC to oversee CDCs' compliance within their AOR. They assist with the movement of non-deployable Service members within their AOR and communicate manning requirements to NAVPERSCOM Deployability Assessment and Assignment Branch (PERS-454).

e. **DES.** The DES is a mechanism for determining fitness for continued naval service, separation, or retirement of Service members because of disability per references (c) through (e). The DES processes include the legacy DES (LDES) and the Integrated Disability Evaluation System (IDES).

f. LIMDU. LIMDU is defined as the identification of a Service member with a certain deployment or Employment limiting medical condition(s). Assignment must be made to an area where required medical care is available.

(1) Temporary Limited Duty (TLD). A Service member is assigned a TLD status when a medical officer expects the Service member to be able to be returned to medically unrestricted duty in a reasonable period of time (12 months or less and based on the medical condition(s)). Service members assigned to a TLD status will be placed in an appropriate DCAT code and their projected rotation date (PRD) will be extended, if necessary, to correspond with the expiration of TLD date.

(2) Permanent Limited Duty (PLD). PLD is the continuation on active duty of a Service member determined unfit for continued naval service by the PEB in a LIMDU capacity. A Service member may request a waiver for PLD under reference (c). If approved the Service member will remain on active duty for a specified time. Service members approved for PLD must be placed in an appropriate DCAT with a PRD that corresponds with the approved PLD date. Once placed in a PLD status, the member may remain at the current command or be assigned to a valid billet as directed by fleet manning control authority priorities.

g. NAVMED 1300/3 Medical Assignment Screening (MAS). MAS is a short and concise medical screening form to specifically review a Service member's medical condition(s) and determine whether a Service member is world-wide assignable. This will be completed after completion of a PEB or at the medical retention determination point portion of the Medical Evaluation Board (MEB).

h. MEB. A MEB is a panel of providers attached to an NMRTC with a commander or commanding officer (CO) designated to hold "convening authority" (CA) for LIMDU assignment. MEBs make recommendations to the CA for determining whether a member needs assignment to LIMDU, recommended for Employ Program, or be referred to the DES.

i. Operational Screening (Sea Duty Screening). After a LIMDU Service member is found fit for medically unrestricted duty and is in receipt of permanent change of station (PCS) orders to an operational command, the Service member must complete operational screening per MILPERSMAN 1300-800 and overseas screening per MILPERSMAN 1300-306, as applicable.

j. **PEB.** The PEB determines the fitness of Service members with medical condition(s) to perform their military duties. For Service members determined unfit because of duty-related condition(s), their eligibility for disability benefits will be reviewed. If the PEB determines an active duty Service member is fit for duty or a Reserve Component Service member is physically qualified, the Service member will return to his or her normal duty or reserve status unless separated or retired on a non-disability basis. A Service member will be considered unfit when a preponderance of the evidence establishes the member, due to disability, is unable to reasonably perform duties of his or her office, grade, rank, or rating, including duties during a remaining period of reserve obligation. A Service member may also be considered unfit when a preponderance of the evidence establishes that:

(1) The Service member's disability represents a decided medical risk to the health, welfare, or safety of the member or other members; or

(2) The Service member's disability imposes unreasonable requirements on the military to maintain or protect the Service member.

k. **Waiver to Remain Onboard.** A request from the member to remain onboard an operational command during the period of LIMDU. The waiver must be submitted to the cognizant detailer for officers or PERS-454 for enlisted members. The waiver must include endorsements from the Service member, the member's CO or officer in charge or (OIC), and the health care provider (HCP) managing the limiting condition(s). The CO or OIC endorsement must be on command letterhead. Requests must be submitted to PERS-454 by e-mail at pregavail-tld.fct@navy.mil.

NOTE: A WAIVER IS NOT REQUIRED FOR LIMDU LESS THAN 90 DAYS.

l. **Employ.** A program that allows for PERS-454 to make a retention determination for a Service member with a prolonged (12 months or more) deployment-limiting condition, but who is still capable of completing an assignment in rate or rating and in a shore duty capacity.

3. Policy

a. Approval of LIMDU Requests. The NMRTC CA approves all LIMDU requests less than 12 consecutive months when a Service member is expected to return to a medically unrestricted duty status at the completion of the LIMDU period. Periods of LIMDU will be condition-based, meaning the length of the LIMDU period will correspond with the treating provider's estimated timeframe for the Service member to recover and return to medically unrestricted duty.

b. Conclusion and Expiration of LIMDU. With the exception of cases referred to a PEB, LIMDU concludes when a Service member is returned to medically unrestricted duty by a MEB or upon expiration of LIMDU. If a Service member's medical LIMDU status is not updated prior to the expiration of LIMDU, PERS-454 will change the Service member's DCAT code to reflect that the Service member's LIMDU is expired, and the Service member may be considered deployable and eligible for orders.

c. Referral to Employ. The NMRTC provider may nominate a Service member for the Employ Program if the Service member is expected to complete the requirements of a non-operational tour. Communities that require special duty screenings must complete their screening and de-screening, and disqualification and force conversion (as applicable), per [MILPERSMAN 1440-011](#), prior to being considered for Employ. Employ Sailors must be returned to duty using [NAVMED 6100/6](#) and receive accounting category code 100 orders (with a DCAT denoting assignment limitation) into valid, funded positions for a minimum of 24 months.

(1) A Service member must complete a re-evaluation for Employ by 15 months prior to his or her Employ tour PRD.

(2) At the re-evaluation, a Service member may be found fit for full duty, referred for an additional period of Employ, or referred to the DES.

(3) Service members who return to full duty or world-wide assignability during their Employ tour will receive a medical assignment screening at re-evaluation.

(4) If a Service member is no longer capable of performing in rating, he or she will be referred to the DES.

d. Referral to DES. Per reference (e), when the course of further recovery is relatively predictable or within 1 year of diagnosis, whichever is sooner, medical authorities will refer eligible Service members into the DES who have one or more medical conditions that may, individually or collectively, prevent the Service member from reasonably performing the duties of his or her office, grade, rank, or rating, including those duties remaining on a reserve obligation for more than 1 year after diagnosis that represent an obvious medical risk to the health of the member or to the health or safety of other members, or that impose unreasonable requirements on the military to maintain or protect the Service member. In all cases, competent medical authorities will refer eligible Service members who meet this criteria within 1 year of diagnosis into the DES.

e. Fit Finding by PEB. Service members processed through the DES and retained in the Military Services due to being found fit for continued naval service by the PEB must be placed in an appropriate DCAT and undergo all required screenings. A Service member may be referred back through the DES to a PEB if the condition(s) for which the Service member was found fit worsens or another condition is identified that meets the criteria listed in reference (e).

f. Retention Waiver for Non-Deployability. Service members who are in a LIMDU status for greater than 12 consecutive months must be reviewed for retention by PERS-454. Recommendation for retention will be based on the Service member's ability to perform appropriate military duties commensurate with his or her office, grade, rank, rate, rating, or skill and the likelihood the Service member's medical condition(s) will allow him or her to return to medically unrestricted duty. When the medical condition(s) will not allow the Service member to return to medically unrestricted duty, the Service member must be directed to a PEB, recommended for Employ, or be recommended for administrative separation per reference (b).

g. Request for Separation, Retirement, or Transfer to Fleet Reserve. LIMDU will not prohibit personnel from requesting separation, retirement, or transfer to the Fleet Reserve.

(1) LIMDU will not delay personnel with an approved separation, retirement, or request for Fleet Reserve transfer. Personnel pending separation or retirement will not be delayed unless approved by NAVPERSCOM Officer Performance and

Separations Branch (PERS-834) (for all legal or performance-flagged officer records); Officer Retirements Branch (PERS-835) (for officer retirements); Distribution Policy and Procedures Branch (PERS-451) (for officer separations); or Bureau of Naval Personnel (BUPERS) Enlisted Military Community Management Division (BUPERS-32) (for enlisted personnel); or other codes as appropriate. Approval may be considered if a Service member is either hospitalized or referred or accepted to the DES for disability processing prior to the release or retirement date per reference (b) and [MILPERSMAN 1830-030](#).

(2) An officer with a pending statutory retirement or separation date deemed physically unfit by a military physician must request in writing to defer his or her statutory date. The request is sent to Assistant Secretary of the Navy (Military & Reserve Affairs (ASN (M&RA))) via PERS-835 for retirements or via PERS-451 for separations per reference (d), section 640.

h. LIMDU Central Coordination. PERS-454 is the central coordination point for the placement and assignment of LIMDU personnel. The function of this central coordination point is to distribute LIMDU personnel. Upon receipt of availability, PERS-454 will review placement and assignment options with input, as needed, from RDCs. A Service member assigned to TLD will be assigned by NAVPERSCOM based on the following guidelines:

(1) TLD is authorized locally by the responsible CA. PERS-454 will review all cases requiring additional TLD in excess of 12 consecutive months.

(2) Service members are assigned within a market capable of providing the required medical care.

(3) In addition to PCS costs and personnel requirements, other factors considered in making assignments include access to care, Bureau of Medicine and Surgery (BUMED) work category code, location of dependents, Service member's past type duty, and physical restrictions imposed by a MEB.

(4) For enlisted personnel: prior to recommending assignments to the assignment control authority (ACA) (the member's detailing team), NAVPERSCOM will consider the number of personnel assigned to an activity, readiness impact, and the distribution of TLD personnel in a geographical area per reference (f).

(5) Activity capacity limits in MyNavy Assignments will be observed. Due to fluctuations in the LIMDU and DES population, the number of LIMDU and DES Service members in a major home port area may temporarily exceed the aggregate capacities of the commands within that area. Commands may request additional LIMDU personnel beyond the approved command capacity with concurrence from the allocation manager's, fleet readiness integrator, TYCOM, and budget submitting office.

4. Roles and Responsibilities

a. BUMED

(1) Ensure all TLD periods and DES cases are entered into the Medical Readiness Reporting System (MRRS), the current TLD computer tracking system (e.g., LDES and SMART), Veterans Tracking Application (VTA), or replacement systems as required.

(2) Collaborate with PERS-454 and other stakeholders in the development of all TLD and Employ training programs per reference (b).

b. BUPERS Military Community Management Department (BUPERS-3). Ensures community management involvement and coordination with NAVPERSCOM for billeting and assignment of Service members through Employ on a case by case basis.

c. NAVPERSCOM

(1) PERS-454

(a) Direct the cognizant NMRTC CA to place a Service member in a TLD status, direct a Service member's case be entered into the DES and referred to a PEB, recommend Employ, or recommend administrative separation for a condition not amounting to a disability (CnD) per reference (b);

(b) Develop and implement the use of DCAT codes for system input, tracking, and reporting of Service members' deployability status;

(c) For enlisted Service members found fit for full duty and WWA following a period of TLD, submit an availability via the Navy Standard Integrated Personnel System (NSIPS) per [MILPERSMAN 1306-1702](#);

(d) Ensure timely and accurate accounting category code (ACC) and DCAT code assignments are made for officer and enlisted Service members when placed in or removed from a TLD status;

(e) Make appropriate NSIPS entries and DCAT code changes for all Service members requiring changes to ACC 105 or from ACC 105 to ACC 100 when applicable;

(f) Ensure Service members assigned TLD while on a normal shore tour (NST) are retained on board with an ACC change to 105 via NSIPS; and

(g) Ensure Service members' PRDs are adjusted as needed based on proposed LIMDU expirations.

(2) Officer Detailers

(a) When approved by PERS-454, assign officers to duty compatible with the physical limitations dictated by the LIMDU AMEBR form;

(b) PERS-454 will assign appropriate DCAT codes and update the officers' ACC from 100 to 105 in NSIPS. The PRD will only be adjusted for those assigned to an NST when the TLD expiration date exceeds the PRD; and

(c) Ensure officers detach from types 2, 3, and 4 duty or type assignment codes (TAC) C and D duty. PCS orders will be issued by the assignment officer detaching a officer from present duty (ACC 100 in most cases) and direct officer to report to duty ashore in a TLD status (ACC 105 and appropriate DCAT). The officer will be assigned to a valid billet if available. The officer's PRD will be set to the month and year at the approved TLD period's expiration as indicated by PERS-454. If the officer has been issued PCS orders prior to LIMDU or if medical treatment is available at the next duty station, the officer may execute the PCS orders. PERS-454 will assign an appropriate DCAT code to reflect LIMDU status. ACC will be managed by PERS-454 for officers who do not require orders.

(d) Ensure officers under the Employ Program receive ACC 100 orders into valid billets at type 1 or type 6 commands for a minimum of 24 months.

NOTE: Any deviation from policy requires PERS-454 approval.

(3) Enlisted Detailers

(a) When approved by PERS-454, the Service member will be assigned to duty compatible with the limitations dictated by the LIMDU AMEBR.

(b) Ensure enlisted Service members currently assigned to shore duty remain at current shore duty stations. PCS orders are not required. PERS-454 will assign the appropriate DCAT code, change the Service member's ACC from 100 to 105 in NSIPS, and adjust the PRDs for those assigned to an NST when the TLD expiration date exceeds the PRD. Nuclear and submarine Service members will be handled per subparagraph 5e of this article.

(c) For Service members assigned to operational duty, PERS-454 will coordinate with NAVPERSCOM Enlisted Distribution Division (PERS-40) to assign them to a shore activity based on the work category code noted on the YH (a class "Y" availability to place member on TLD or refer to PEB) availability received from the losing command (see subparagraph 4e(12) below for more information). If the Service member has been issued PCS orders prior to LIMDU or if medical treatment is available at the next duty station, the Service member may execute the PCS orders.

(d) Ensure Service members under the Employ Program receive ACC 100 orders into valid billets at type 1 or type 6 commands for a minimum of 24 months.

NOTE: Any deviation from policy requires PERS-454 approval.

d. NMRTC

(1) Develop steps to ensure assignment of TLD periods are based on BUMED's recommended recovery period for the specific medical condition. Condition-based LIMDU duration must not exceed 12 consecutive months;

(2) Enter all TLD cases and associated work category code into SMART or the replacement system and all DES cases into VTA. Update member's status in MRRS;

(3) Assist CDCs and TLD Service members with obtaining appointments on a priority basis;

(4) Establish local procedures to ensure Service member reports to the NMRTC patient administration office immediately upon recommendation for placement on, extension of, or removal from TLD;

(5) Upon completion of the [NAVMED 6100/5](#), immediately inform the Service member's parent command that the Service member is to be processed for TLD and include the Service member's work category code. Ensure MRRS and SMART are updated within 24 hours for PERS-454 visibility following CA approval;

(6) Ensure MDC conducts monthly meetings with CDCs to review current cases, discuss potential problems, and analyze existing processes;

(7) Provide training to CDCs and view access to SMART, MRRS, and relevant programs;

(8) Provide assistance to CDCs when TLD Service members are unable to schedule appointments;

(9) Provide guidance to medical providers assigning TLD. At any time during a period of TLD, the attending medical officer may make one of the following determinations:

(a) Service members will be able to return to medically unrestricted duty;

(b) Nominate Service members for Employ who are non-deployable, but capable of completing an assignment in rating in a shore capacity;

(c) Service member requires additional TLD (TLD exceeding 12 consecutive months requires PERS-454 approval), or

(d) Service member requires referral into the DES.

(10) Ensure all cumulative TLD periods of 12 consecutive months or less are approved by the cognizant NMRTCs CA;

(11) When TLD cases are returned to medically unrestricted duty or approved for Employ, the AMEBR requires signature from the cognizant NMRTC CA. NMRTC CA signing of

[NAVMED 6100/6](#) must be completed no more than 5 working days from the date a Service member was returned to medically unrestricted duty;

(12) Ensure providers make return to medically unrestricted duty determinations and recommendations of additional TLD or DES referral no later than 30 days prior to the expiration of TLD;

(13) Ensure Service members on TLD complete face-to-face medical reevaluation 30 days prior to the end of their TLD periods and determine one of the following:

(a) The Service member is able to return to medically unrestricted duty,

(b) The Service member is recommended for an additional period of TLD (not to exceed 12 months consecutively),

(c) The Service member requires referral into the DES, or

(d) The Service member requires administrative separation processing for a CnD.

(14) If a Service member is recommended for additional periods of TLD that exceed 12 consecutive months, forward the case via SMART to PERS-454 for review, waiver consideration, or other action;

(15) Ensure the MDC reports all TLD and DES cases in MRRS and SMART; and

(16) Comply with all PERS-454 directives to place Service members in a TLD status, referral to the DES, and or recommendation for administrative separation processing within 30 days of notification. Any additional information or adjudication appeal requests must be submitted to PERS-454 within 30 days. An appeal request must be accompanied by a copy of the [NAVMED 6150/50](#) Naval Disability Evaluation System Medical Evaluation Board Report that has been signed by a convening authority indicating that a PEB is not warranted.

e. Command

(1) Fully integrate TLD Service members into all aspects of the command to the maximum extent practical. COs and OICs remain responsible to facilitate and ensure LIMDU Service members attend all of their necessary medical and administrative appointments, which is their primary mission. As such, leadership must be thoughtful and deliberate in assigning duties which may be completed between appointments, actively support the command's mission, and reinforce the importance of each Service member to the command.

(2) Ensure TLD Service members report for all scheduled appointments and remain available for all follow-up care. Comply with the following guidelines:

(a) TLD Service members will not be assigned on temporary additional duty or temporary duty (TDY) outside the geographic area without medical clearance from the HCP managing the LIMDU condition(s).

(b) Coordinate the scheduling of appointments with TLD Service members and cognizant NMRTC.

(c) For Service members on TLD, ensure leave requests (other than emergency leave) are coordinated with the CDC and do not conflict with medical appointments.

(3) Monitor Service members in temporary or permanently non-deployable status and maintain close liaison with their respective command career counselors on issues of expirations of hard and or soft end active obligated service (EAOS/SEAOS) or mandatory separation while on TLD, referral to the DES in line with reference (b). Contact BUPERS-32 for enlisted personnel and PERS-451 for officers for guidance concerning Service members with an expired SEAOS or mandatory separation date during a period of TLD, DES, or PEB.

(4) Meaningfully Employ Service members based on their capabilities, medical restrictions, and work category code restrictions while assigned on TLD.

(5) Designate a CDC to function as liaison for the Deployability Program. Commands with 50 or more LIMDU personnel will appoint a CDC as a primary duty and assign collateral CDCs

on a 1:50 ratio to assist in the management of non-deployable personnel. CDCs will be trained by the local MDC.

(6) Commands will document expectations of LIMDU Service members via [NAVPERS 1070/613](#) that outlines the following: period of LIMDU, point of contact at the NMRTC, responsibility to adhere to treatment plan, retain close contact with the CDC, prohibition of elective surgery without prior authorization, and that failure to comply with LIMDU recommendations and plan may constitute a violation of the Uniform Code of Military Justice.

(7) Upon Service member's hospitalization, maintain close liaison with the NMRTC and hospital. If required, TDY orders may be issued for a period of hospitalization. If PCS orders are required, contact PERS-454 for coordination after initiation of LIMDU.

(8) Assist Service members in scheduling necessary appointments prior to expiration of TLD by coordinating with the MDC. The parent command deployability coordinator is responsible for obtaining TLD members' medical evaluation status no later than 30 days prior to the expiration of TLD.

(9) For Service members under orders who are subsequently placed on TLD, submit an order cancellation request message to the applicable detailer if follow-on assignment is not compatible with physical limitations or near an NMRTC.

(10) For Service members found fit for duty by the PEB, ensure the completion of a MAS (documented on [NAVMED 1300/1](#) Medical, Dental, and Educational Suitability Screening) when applicable. A MAS is required upon a determination of fit for continued naval service by the PEB. This action must be completed within 15 days of a PEB finding a member fit for duty. Submit an "assignment limited" message to PERS-454 within 24 hours of MAS completion. "Fit for duty" findings without any assignment limitations must be submitted to the servicing transaction service center (TSC).

(11) For Service members nominated for the Employ Program, the CO or OIC will complete the [NAVPERS 1300/35](#) Employ Candidate Assessment within 10 working days and submit to PERS-454 for final determination. For Service members assigned to the command on Employ orders, ensure that re-evaluations are completed no later than 15 months prior to the end of the Employ tour PRD.

(a) For enlisted Employ Service members, command career counselors will submit a [NAVPERS 1306/7](#) Electronic Personnel Action Request to ensure obligated service requirements of the assigned billet are met prior to execution.

(b) Forced-conversion, lateral conversion, or re-designation is allowed while serving on an Employ tour per [MILPERSMAN 1440-010](#) and [1440-011](#).

(12) Submit a message report for Service members assigned to overseas duty (type duty codes 3, 4, or 6 or TACs C and D) to the appropriate ACA (detailing team), copy PERS-454, comment on the availability of required medical care at the overseas activity and on the Service member's abilities to be used effectively on board with current condition(s) during the TLD period.

(a) If required medical care is available and the Service member's condition does not prevent him or her from being used productively on board during the period of TLD, comply with procedures identified for Service members assigned to shore duty (type duty code 1 or TAC S).

(b) For officers, if required medical care is not available or the Service members' current condition(s) prevent their effective use on board during the period of TLD, commands will submit an early return request via BUPERS Online (BOL) and coordinate with their detailers for orders as per [MILPERSMAN 1300-306](#).

(c) For enlisted personnel, if required medical care is not available or the Service members' current condition(s) prevent their effective use on board during the period of TLD, commands will submit a YH availability report and an "early return" request via BOL as per [MTI.DPDCSMAN 1306-1702](#) and [MILPERSMAN 1300-306](#).

(13) For enlisted personnel, ensure YH availabilities, submitted via e-mail to pregavail-tld.fct@navy.mil, contain the following information:

(a) BUMED work category code and any additional physical or geographical limitations;

(b) Family members' location, if applicable;

(c) For overseas locations, include in remarks whether local treatment facilities may provide adequate medical care or submit an early return request via BOL; and

(d) (Optional) any special circumstances (e.g., Service member's current household goods status.) or whether the command would like to recommend a specific unit for the Service member's assignment.

(14) For nuclear-trained enlisted personnel, complete requirements and forward documentation. The Service member's command will ensure the member completes all medical requirements needed to determine fitness for duty onboard submarines and or fitness for duty involving exposure to ionizing radiation and that documentation is forwarded to the command personnel office. Screening is completed by the Office of the Chief of Naval Operations (OPNAV) Nuclear Enlisted Programs Branch (OPNAV N133D) for nuclear-trained operators. Nuclear-trained surface and nuclear-trained submarine Service members must not be made available from LIMDU until screened by OPNAV N133D prior to expiration of their LIMDU period per reference (g).

(15) For nuclear-trained enlisted community:

(a) Required Notification for Examinations and or Screening. The CDC or personnel office for a Service member assigned to LIMDU will ensure the Service member and command are aware of all required physical examinations and or screening in time to be completed prior to the expiration of the LIMDU period per reference (g).

(b) Documentation to OPNAV N133D for Nuclear-Trained Operators. The command CDC or personnel office submitting the availability for assignment to a LIMDU status on a nuclear-trained surface or nuclear-trained submarine member must forward a copy of the medical board report, with enclosures, to OPNAV N133D by separate correspondence.

f. TSC

(1) Ensure regular gain and travel transactions are processed for all Service members with LIMDU orders;

(2) If Service members are referred to the PEB, their commands will forward all final PEB disposition paperwork to TSC upon receipt of PEB disposition documentation from Service member's command;

(3) Ensure timely and accurate ACC assignments are made for officer and enlisted Service members when they are placed in or removed from a PEB status;

(4) Enlisted Service members found fit for continued naval service by the PEB without assignment limitations must complete a MAS and submit it to TSC via Salesforce. Once completed, a YJ availability report (a class "Y" availability) will be submitted through NSIPS per [MILPERSMAN 1306-1702](#), and

(5) Make appropriate NSIPS entries for all Service members gained in or changed to PEB ACC 355.

g. Service Member

(1) Report to the NMRTC Patient Administration Office immediately upon being recommended for placement, extension of, or removal from LIMDU;

(2) Attend all required medical appointments. All medical appointments for a Service member in a LIMDU status must be attended as directed by the medical officer. Any time there is a change in LIMDU status, the Service member must report it to his or her local chain of command and the CDC within 24 hours; and

NOTE: Missing a medical appointment may be punishable under the UCMJ per reference (h).

(3) If the Service member is executing PCS orders and becomes medically non-deployable en route, the gaining command and Service member (if able) must contact the originating detailee and PERS-454 for an order modification.

(4) Employ Service members must complete re-evaluation for Employ no later than 15 months prior to the end of their Employ tour PRD. Enlisted Employ Service members who will exceed their EAOS during the course of their Employ tour must oblige by submitting either a retain in Service waiver or executing a short-term contract extension through their command

TSC and, if applicable, high year tenure (HYT) waiver through the PRD of the orders they are set to execute. BUPERS-328 is the action officer in either case.

5. Procedures

a. NMRTC. When a Service member is determined by the cognizant NMRTC to be able to return to medically unrestricted duty, no more than 5 working days may elapse between return to medically unrestricted duty, CA signature, and notification to the CDC. All determinations for return to medically unrestricted duty require CA signature. If a Service member was found fit for continued naval service by the PEB, the parent command will ensure the Service member completes an MAS within 15 days of the PEB findings. If there are no medical limitations, forward completed assignment screening to the TSC within 24 hours. If there are medical limitations, forward the MAS to PERS-454.

b. TSC. The TSC will ensure LIMDU gain and travel transactions are processed for all Service members with ACC 105 orders.

(1) For Service members assigned to shore duty, TSC will change ACC from 100 or 105 to 355. PERS-454 will assign appropriate DCAT code. See subparagraph 5e of this article for nuclear and submarine personnel assignment.

(2) For Service members in receipt of orders, TSC will change ACC from 100 or 105 to 355. For officers, the ACC will not be changed to ACC 105 until ACC 355 is available.

(3) Upon receipt of a completed AMEBR or return of Service member to fit for full duty, TSC will change ACC from 355 to 100.

c. PERS-454

(1) Assignment of Members Returned to Medically Unrestricted Duty. Service members, assigned to or who are currently on shore duty, will have the period of LIMDU applied to their NST. If the LIMDU is completed before expiration of their assigned PRD, the PRD will not be adjusted. Post-LIMDU assignments will follow normal sea/shore flow.

(2) Assignment-Limited Service Members. Service members found to be "assignment-limited" after the PEB will be assigned the appropriate DCAT code by PERS-454 after review of the provided MAS.

(3) Personnel in a PLD Status. PERS-454 will place Service members retained after being found unfit by the PEB in ACC 105.

(4) Personnel With Approved Retirement or Fleet Reserve Dates. For Service members able to return to medically unrestricted duty and who have approved retirement or Fleet Reserve dates or are beyond HYT, no availability is required. PERS-454 will assign an appropriate DCAT code and change ACC to 100.

(5) Assignment of Members on Employ Orders. PERS-454 will provide the final determination for Employ nominations. Re-evaluation notification via message will be released from PERS-454 at 18 months prior to the Employ PRD.

d. Early Separation. Refer to [MILPERSMAN 1910-216](#) and [MILPERSMAN 1910-218](#) as appropriate.

e. Policy for Enlisted Nuclear-Trained and Submarine Service Members on LIMDU. Nuclear-trained surface, nuclear-trained submarine, and non-nuclear-trained submarine Service members require additional screening and administrative processing for assignment to and from a LIMDU status. All the above Service members, regardless of sea/shore type duty code or type assignment code, must be made available (class YH availability) for assignment to LIMDU with a PRD established for the length of the LIMDU period.

(1) Procedures. To eliminate delays in returning Service members to full duty, appropriate actions must be immediately executed and meticulously tracked to ensure all requirements are completed in a timely manner.

(2) Medical Evaluation. All nuclear-trained surface, nuclear-trained submarine, and non-nuclear-trained submarine Service members detached for hospitalization or LIMDU must be expeditiously evaluated by an undersea medical officer to determine potential for continued service in the nuclear field or Submarine Force. If nuclear field or submarine

disqualification is appropriate, the parent command must submit the disqualification finding and [NAVPERS 1221/6](#) Navy Enlisted Classification Change (NEC) Request per reference (i).

(3) Inquiries. Inquiries related to screening status or requirements for all nuclear-trained enlisted personnel should be directed to OPNAV N133D.

6. Applicable Forms

a. [NAVMED 1300/3](#). A medical administrative document for active duty Service members to return to medically unrestricted duty or identify assignment limitations after a PEB. This screening is for Navy active duty Service members only and requires complete demographic information including name, rank or grade, and Social Security number.

b. [NAVMED 6100/5](#). An abbreviated report used to assign Service members to TLD and to refer Service members to a PEB. This form is found and completed electronically in the SMART System.

c. [NAVMED 6100/6](#). Used to return a Service member to medically unrestricted duty after a period of LIMDU and to identify any duty or assignment limitations.

d. [NAVMED 6150/50](#). This report is used to determine whether a Service member's condition warrants a referral to the DES.

e. [NAVPERS 1300/16](#). Report on Suitability for Overseas Assignments. Page 3 is used to identify medical suitability of a Service member.

f. [NAVPERS 1221/6](#). Removes or changes the NECs that identify enlisted personnel trained as nuclear propulsion plant operators and supervisors.

g. [NAVPERS 1300/35](#). Used to determine a Service member's professional fitness for the Employ Program. The form is to be completed by the CO or OIC and utilized by detailers to determine a Sailor's order location.